

CONTROLLED KEY-REPLY EVIDENCE EXTRACT

Professor Philip Asherson - ADHD continuity, active risk-management reasoning, review/restart next step and quotation permission

Source account	matt.bayliss99@gmail.com
Gmail thread ID	1a03135d47df5139
Key exchange	24-27 August 2026
Evidence status	QUOTABLE - explicit permission obtained

Control note. This is a focused evidence extract, not a claim that Professor Asherson adjudicated the individual case. His stated limitations are preserved: he was not involved in the care, lacked the full clinical details/local service information, and was not then in clinical practice. The Gmail thread remains the authoritative source.

24 Aug 2026 - Professor Asherson: continuity principle

Gmail message ID: 1a032e92460ea3b9

I can't address the specific responses outlined in your document. However, I will cite NICE guidelines on what is expected.

NICE specifically addresses continuity of care in ADHD in guideline NG87, "Attention deficit hyperactivity disorder: diagnosis and management."

The key recommendation is: "Healthcare providers should ensure continuity of care for people with ADHD." - NICE NG87, recommendation 1.5.1.

Importantly, the general recommendation 1.5.1 is not restricted to children or to transition: it says people with ADHD and healthcare providers should ensure continuity of care. That could therefore be relevant when considering whether an adult ADHD service can appropriately discharge or transfer someone without arrangements for ongoing care.

26 Aug 2026 - Professor Asherson: explicit limitations and general clinical view

Gmail message ID: 1a03eb17632bb464

Dear Matthew,

First to clarify I can make only general comments as I am not involved in your case, lack clinical details, and not currently in clinical practice. I also have no information on how local services are organised in your area.

From your notes, the problem regarding continuity appears to have arisen because you withdraw from the service in December 2025. No alternative arrangements were in place at that time so effectively you were no longer under the care of a specialist service. Note, this is often considered a requirement for safe prescribing.

I cannot comment on the deterioration - whether related to ADHD or any other problem. What you describe are not typical symptoms of ADHD so could reflect a different process. This would need to be evaluated and not only ADHD.

In general medication for ADHD that is effective should be continued.

I do not know the arrangements for prescribing in your region, but NICE do recommend shared care with primary care. These arrangements often include the need to be under the care of a Specialist service - often the local specialist NHS ADHD service.

The problem might be uncertainty over the nature of the new symptoms you describe and the potential cause of these.

There may also be no fast-track process in place, back into a service once discharged. NHS resources are often limited, so there may be no other specialist clinician available within the local ADHD service.

I hope you will be reviewed shortly, as this does seem to be the next important step.

27 Aug 2026 - professional reasoning put to Professor Asherson after focused records

Gmail message ID: 1a0407c1a759dded

Matthew supplied three focused primary records and asked a narrow professional question. The reasoning put to Professor Asherson was that, where a specialist psychiatric service has foreknowledge of established effective ADHD treatment and subsequent evidence of deterioration and foreseeable risk, good psychiatric practice and clinical governance would ordinarily require active risk ownership - timely specialist review, a documented risk-benefit decision, appropriate neurology/cardiology coordination, and a safe continuity/interim plan - rather than simply leaving the person untreated on an ordinary waiting pathway. The question also asked whether atypical motor symptoms make cross-specialty coordination more important when those symptoms form part of the treatment decision.

Matthew expressly stated that he was not asking Professor Asherson to determine causation, misconduct or disputed individual facts.

27 Aug 2026 - Professor Asherson replies to that reasoning

Gmail message ID: 1a041c71395b6d5d

Based on the information you have given me, your reasoning is correct.

Philip

27 Aug 2026 - Matthew defines scope and requests quotation permission

Gmail message ID: 1a041e967b3b9b27

To avoid overstating your brief reply, I understand it to endorse the general professional reasoning set out in my preceding email: where a specialist psychiatric service has the described foreknowledge and subsequent evidence of deterioration and foreseeable risk, good practice and clinical governance would ordinarily require active risk ownership through timely specialist review, a documented risk-benefit decision, appropriate neurology/cardiology coordination, and a safe continuity or interim plan; and the presence of atypical motor symptoms makes cross-specialty coordination more important where those symptoms form part of the treatment decision.

I do not understand your reply as determining causation, misconduct, or disputed individual facts, or as replacing assessment by the clinicians responsible for my care.

Please confirm whether the paragraph above accurately records the scope you intended. Separately, please confirm whether I may quote, in private complaint, regulatory and advocacy correspondence and always alongside your stated limitations: "In general medication for ADHD that is effective should be continued." and "Based on the information you have given me, your reasoning is correct."

27 Aug 2026 - Professor Asherson grants permission and identifies next step

Gmail message ID: 1a041f24a6a7c61d

Yes. You may quote me.

Clearly the next step is for review and restart treatment for ADHD if indicated after a risk-benefit analysis. I note you were already requested to check cardiovascular status, presumably with this in mind.

EVIDENTIAL USE NOTE

"In general medication for ADHD that is effective should be continued."

"Based on the information you have given me, your reasoning is correct."

"Clearly the next step is for review and restart treatment for ADHD if indicated after a risk-benefit analysis. I note you were already requested to check cardiovascular status, presumably with this in mind."

Permission: Professor Asherson explicitly wrote, "Yes. You may quote me."

What this supports: a general professional continuity/risk-management proposition; the need for review and a reasoned risk-benefit decision after the requested cardiovascular status was checked; and the reasonableness of the active-risk-ownership/cross-specialty-coordination reasoning as framed in the immediately preceding exchange.

What it does not support: a finding that CWPT committed negligence, discrimination, misconduct or a legal breach; a conclusion that dexamfetamine caused or treats every motor symptom; or a patient-specific prescription order. Professor Asherson repeatedly limited himself to general professional comments.

Source-control rule: preserve these limitations whenever the quotations are used. Where the question is "who should now resolve the treatment problem?", the strongest exact wording is that the next step is review and restart if indicated after risk-benefit analysis, with the cardiovascular-status prerequisite specifically noted as already requested.